

Subject: Re: Relationship
Date: Sunday, December 12, 2021 at 9:29:52 AM Eastern Standard Time
From: Ryan D
To: Michelle Kohn
Attachments: image001.jpg, image002.png

Would you call me today it's important. Or respond to my email?

From: Michelle Kohn <michelle@dreamsarerealty.com>
Sent: Sunday, July 18, 2021 5:31 PM
To: ryand1981@hotmail.com <ryand1981@hotmail.com>
Subject: Relationship

First of all, and I know I have said this many times before, but you have to truly want to change for yourself. Nothing I can say or do will help you if you don't TRULY want this for yourself. Anyone can change temporarily but if they don't want it for themselves, they revert to old behaviors and patterns which is what has always happened in the past. And before you get defensive, I know I have things I need to change also. I'm actually very well aware of the things I need to change but I'm also very aware that I will never be that person with you if many things don't change.

We have also discussed the issue of trust before, and I explained how that applies to a relationship in MANY different ways. What happened was very scary for many reasons! The biggest reason is because I love you and want to be with you, I always have and if you're being honest with yourself, you know that's true. I want a future with you but when you get that angry and violent it makes me feel like we can't have a future together because I can't be in a relationship where that EVER happens, and it scares the hell out of me because I want to be married and have a family, but I only want to do it ONCE! How am I supposed to trust that you're going to take care of me and a family if you're behaving like that? As I have said before, I will NEVER expose my children to that kind of behavior!

You become a Completely different person when you're in a rage like that. When I looked in your eyes, it was like you were a completely different person and there was absolutely nothing I could say or do to bring you back. There was no way to reason with you or have a conversation. You made your mind up on how you were feeling and what the reality of the situation was and acted based on that without even attempting to discuss it and see or understand how I was feeling or what even happened. You said you couldn't even remember what happened but that didn't stop you from the impulsive and violent behavior.

People can change if they truly want to... who we were in previous relationships doesn't determine who we are in the next one.

Regardless of what you say about who you were or things you did in previous relationships, that doesn't apply in this relationship. In fact, your previously relationships could have and probably did contribute to issues in ours. And I'm not saying that doesn't apply to me as well, but I do know who I am now and how I want to be in a relationship. I'm not sure if everything can be fixed but I do know that it will never work if we don't both change at the same time.

And speaking of time...we are almost out of time. We are not getting any younger. There comes a point where we either have to make things right and move forward OR we finally decide to move on. Unfortunately, every time you have a blow up it delays even the potential of fixing things again! I'd be lying if I said I don't get more and more upset every time this happens thinking about the fact that it's just cost us so much more time and then I'm stuck with deciding if I want to spend more time waiting and hoping that it can work, but with the fear in the back of my head that it's just MORE wasted time.

I've been doing some research and I am in no way saying that this applies to you, but I would encourage you to consider the possibility that it might. If you do choose to go back and talk to the guy you were talking to before, maybe you could ask him about it. I am only sharing because I truly

want to help you and if there's a possibility that this applies or something similar and we don't figure it out, accept it and figure out a solution, then there is no future for us.

I want to help you if I can, but first you have to acknowledge whatever it is that's going on here and figure out how to fix it before I can help. Reading this has actually been enlightening to me because if we are dealing with something like this, then my reaction to your behavior, right or wrong, is only making the situation worse. So that is something I have to figure out. I know that I shut down at times and I don't want to do that but there are also non-negotiables that can NEVER happen if this is ever going to work. I can't live in fear of the person that I'm supposed to be able to trust that they will be the one person who will always protect me and take care of me.

I hope you can appreciate the fact that I have spent a lot of time doing research today and I am in no way send this to hurt your feelings or accuse you of anything and I am definitely not a doctor. I am only trying to help you and us if there's a chance of there still being an "us".

(And...I think we are both a little OCD)

Reasons to consider-

- Out of Body (Seem like a completely different person and impossible to speak to when in rage)
- Anger can last for several hours to several days
- Shut down for hours to days at a time
- Fear of abandonment
- Trust Issues
- Anger & Aggression- verbal & violent, escalates quickly, intense anger displayed in inappropriate ways
- Impulsiveness
- Emotional Instability- Intense mood swings, loved ones exasperate the situation and make it worse
- Dangerous behavior
- Narrow-minded views- seeing things or people as entirely positive or entirely negative
- Paranoid- feeling everyone is against you or out to get you
- Self Harm (have said that you have had thoughts of it being easier to just "give up")

Potential causes-

- Childhood
- Previous relationships
- Possibly lower levels of CSF 5-HIAA ? (Something I can across online)

Borderline Personality Disorder Symptoms

There are many possible symptoms of BPD, and their frequency and severity can all vary from person to person. Some people may be able to prevent symptoms or keep them at bay by staying away from circumstances that trigger their symptoms, while unavoidable occurrences may trigger others.¹

BPD symptoms include:¹

- Mood swings
- Uncertainty surrounding one's identity
- Unpredictable or erratic shifts in interests, values, or opinions
- Narrow-minded views, such as seeing things or people as entirely positive

or entirely negative

- Unstable relationships with friends, family, and romantic partners
- Fear of abandonment and coping with abandonment fears by impulsively entering into or withdrawing from relationships
- Engaging in impulsive and risky behaviors like abusing substances, lavish spending, unsafe sex, or driving recklessly
- [Self-harming behaviors](#) such as cutting or self-inducing pain
- [Suicidal ideation](#)
- Threatening suicide
- Feeling empty, alone, or disassociated from reality or oneself
- [Intense anger](#) displayed in inappropriate ways
- Trust issues

Holding extreme and volatile views on people or things is sometimes referred to as “splitting.” According to Dr. Reich, splitting describes an inability to “simultaneously hold in mind positive and negative images of yourself and other people,” resulting in views or feelings that are either black or white, all or nothing, or good or bad. For example, someone with BPD may rapidly swing from liking a person to intensely disliking them. “The same thing can happen in relation to images of yourself,” Dr. Reich says. “You may go from feeling sort of OK about yourself to intense self-hatred if you have a borderline personality disorder.”⁹

Two other terms that may come up in relation to BPD are insomnia and hypersexuality. However, according to experts, neither of these symptoms are a core facet of BPD. Unsurprisingly, insomnia is a common and frustrating affliction for some people with BPD, just as it is common in many other mental health conditions. For people with BPD, [insomnia and other sleep problems](#) can exacerbate existing symptoms, like difficulty regulating emotions.¹⁰

Hypersexuality is less common, experts say, but for some people with BPD, compulsive [sexual behavior](#) may be an outgrowth of their impulsivity or unstable relationships. “There’s a certain subset of borderline patients who may use sexual impulsivity as a way of soothing themselves or feeling better,” says Dr. Reich. “But I wouldn’t say it’s a hallmark of borderline personality disorder. There’s certainly some patients who engage in it, but there are plenty who have very impoverished social and romantic lives.”

Borderline Personality Disorder

Overview

Borderline personality disorder is an illness marked by an ongoing pattern of varying moods, self-image, and behavior. These symptoms often result in impulsive actions and problems in relationships. People with borderline personality disorder may experience intense episodes of anger, depression, and anxiety that can last from a few hours to days.

Signs and Symptoms

People with borderline personality disorder may experience mood swings and display uncertainty about how they see themselves and their role in the world. As a result, their interests and values can change quickly.

People with borderline personality disorder also tend to view things in extremes, such as all good or all bad. Their opinions of other people can also change quickly. An individual who is seen as a friend one day may be considered an enemy or traitor the next. These shifting feelings can lead to intense and unstable relationships.

Other signs or symptoms may include:

- Efforts to avoid real or imagined abandonment, such as rapidly initiating intimate (physical or emotional) relationships or cutting off communication with someone in anticipation of being abandoned
- A pattern of intense and unstable relationships with family, friends, and loved ones, often swinging from extreme closeness and love (idealization) to extreme dislike or anger (devaluation)
- Distorted and unstable self-image or sense of self
- Impulsive and often dangerous behaviors, such as spending sprees, unsafe sex, substance abuse, reckless driving, and binge eating. **Please note:** If these behaviors occur primarily during a period of elevated mood or energy, they may be signs of a mood disorder—not borderline personality disorder
- Self-harming behavior, such as cutting
- Recurring thoughts of suicidal behaviors or threats
- Intense and highly changeable moods, with each episode lasting from a few hours to a few days
- Chronic feelings of emptiness
- Inappropriate, intense anger or problems controlling anger
- Difficulty trusting, which is sometimes accompanied by irrational fear of other people's intentions
- Feelings of dissociation, such as feeling cut off from oneself, seeing oneself from outside one's body, or feelings of unreality

Not everyone with borderline personality disorder experiences every symptom. Some individuals experience only a few symptoms, while others have many. Symptoms can be triggered by seemingly ordinary events. For example, people with borderline personality disorder may become angry and distressed over minor separations from people to whom they feel close, such as traveling on business trips. The severity and frequency of symptoms and how long they last will vary depending on the individual and their illness.

Risk Factors

The cause of borderline personality disorder is not yet clear, but research suggests that genetics, brain structure and

function, and environmental, cultural, and social factors play a role, or may increase the risk for developing borderline personality disorder.

- **Family History.** People who have a close family member, such as a parent or sibling with the disorder may be at higher risk of developing borderline personality disorder.
- **Brain Factors.** Studies show that people with borderline personality disorder can have structural and functional changes in the brain especially in the areas that control impulses and emotional regulation. But is it not clear whether these changes are risk factors for the disorder, or caused by the disorder.
- **Environmental, Cultural, and Social Factors.** Many people with borderline personality disorder report experiencing traumatic life events, such as abuse, abandonment, or adversity during childhood. Others may have been exposed to unstable, invalidating relationships, and hostile conflicts.

Although these factors may increase a person's risk, it does not mean that the person will develop borderline personality disorder. Likewise, there may be people without these risk factors who will develop borderline personality disorder in their lifetime.

Treatments and Therapies

Borderline personality disorder has historically been viewed as difficult to treat. But, with newer, evidence-based treatment, many people with the disorder experience fewer or less severe symptoms, and an improved quality of life. It is important that people with borderline personality disorder receive evidence-based, specialized treatment from an appropriately trained provider. Other types of treatment, or treatment provided by a doctor or therapist who is not appropriately trained, may not benefit the person.

Many factors affect the length of time it takes for symptoms to improve once treatment begins, so it is important for people with borderline personality disorder and their loved ones to be patient and to receive appropriate support during treatment.

Tests and Diagnosis

A licensed mental health professional—such as a psychiatrist, psychologist, or clinical social worker—experienced in diagnosing and treating mental disorders can diagnose borderline personality disorder by:

- Completing a thorough interview, including a discussion about symptoms
- Performing a careful and thorough medical exam, which can help rule out other possible causes of symptoms
- Asking about family medical histories, including any history of mental illness

Borderline personality disorder often occurs with other mental illnesses. Co-occurring disorders can make it harder to diagnose and treat borderline personality disorder, especially if symptoms of other illnesses overlap with the symptoms of borderline personality disorder. For example, a person with borderline personality disorder may be more likely to also experience symptoms of depression, bipolar disorder, anxiety disorders, substance use disorders, or eating disorders.

Seek and Stick with Treatment

NIMH-funded studies show that people with borderline personality disorder who don't receive adequate treatment are:

- More likely to develop other chronic medical or mental illnesses
- Less likely to make healthy lifestyle choices

Borderline personality disorder is also associated with a significantly higher rate of self-harm and suicidal behavior than the general public.

People with borderline personality disorder who are thinking of harming themselves or attempting suicide need help right away.

If you or someone you know is in crisis, call the toll-free [National Suicide Prevention Lifeline \(NSPL\)](#) at **1-800-273-TALK (8255)**, 24 hours a day, 7 days a week. The service is available to everyone. The deaf and hard of hearing can contact the Lifeline via TTY at 1-800-799-4889. **All calls are free and confidential.** Contact social media outlets directly if you are concerned about a friend's social media updates or dial 911 in an emergency. Read more on NIMH's [Suicide Prevention](#) health topic page.

The treatments described on this page are just some of the options that may be available to a person with borderline personality disorder.

Psychotherapy

Psychotherapy is the first-line treatment for people with borderline personality disorder. A therapist can provide one-on-one treatment between the therapist and patient, or treatment in a group setting. Therapist-led group sessions may help teach people with borderline personality disorder how to interact with others and how to effectively express themselves.

It is important that people in therapy get along with, and trust their therapist. The very nature of borderline personality disorder can make it difficult for people with the disorder to maintain a comfortable and trusting bond with their therapist.

Two examples of psychotherapies used to treat borderline personality disorder include:

- **Dialectical Behavior Therapy (DBT):** This type of therapy was developed for individuals with borderline personality disorder. DBT uses concepts of mindfulness and acceptance or being aware of and attentive to the current situation and emotional state. DBT also teaches skills that can help:
 - Control intense emotions

- Reduce self-destructive behaviors
- Improve relationships
- **Cognitive Behavioral Therapy (CBT):** This type of therapy can help people with borderline personality disorder identify and change core beliefs and behaviors that underlie inaccurate perceptions of themselves and others, and problems interacting with others. CBT may help reduce a range of mood and anxiety symptoms and reduce the number of suicidal or self-harming behaviors.

Read more on NIMH's [Psychotherapies](#) health topic page.

Medications

Because the benefits are unclear, medications are not typically used as the primary treatment for borderline personality disorder. However, in some cases, a psychiatrist may recommend medications to treat specific symptoms such as:

- mood swings
- depression
- other co-occurring mental disorders

Treatment with medications may require care from more than one medical professional.

Certain medications can cause different side effects in different people. Talk to your doctor about what to expect from a particular medication. Read more in NIMH's [Mental Health Medications](#) health topic.

Other Elements of Care

Some people with borderline personality disorder experience severe symptoms and need intensive, often inpatient, care. Others may use some outpatient treatments but never need hospitalization or emergency care.

Therapy for Caregivers and Family Members

Families and caregivers of people with borderline personality disorder may also benefit from therapy. Having a relative or loved one with the disorder can be stressful, and family members or caregivers may unintentionally act in ways that can worsen their loved one's symptoms.

Some borderline personality disorder therapies include family members, caregivers, or loved ones in treatment sessions. This type of therapy helps by:

- Allowing the relative or loved one develop skills to better understand and support a person with borderline personality disorder
- Focusing on the needs of family members to help them understand the obstacles and strategies for caring for someone with borderline personality disorder. Although more research is needed to determine the effectiveness of family therapy in borderline personality disorder, studies on other mental disorders suggest that including family members can help in a person's treatment.

Finding Help

More information about finding a health care provider or treatment for mental disorders in general is available on our [Help for Mental Illness](#) webpage.

Tips for Family and Caregivers

To help a friend or relative with the disorder:

- Offer emotional support, understanding, patience, and encouragement—change can be difficult and frightening to people with borderline personality disorder, but it is possible for them to get better over time
- Learn about mental disorders, including borderline personality disorder, so you can understand what the person with the disorder is experiencing
- Encourage your loved one who is in treatment for borderline personality disorder to ask about family therapy
- Seek counseling for yourself from a therapist. It should not be the same therapist that your loved one with borderline personality disorder is seeing

<https://www.nimh.nih.gov/health/topics/borderline-personality-disorder/>

Borderline Personality Disorder DSM-5 Criteria

To be officially diagnosed with BPD, one typically has to meet five or more of the following nine criteria:¹¹

1. Intense fear of abandonment leading one to take frantic or extreme measures to avoid feeling or being abandoned
2. Continuously engaging in unstable and intense relationships and experiencing shifting and extreme feelings toward people (like switching between loving and hating someone)
3. Uncertain and unstable self-image or sense of self
4. Engaging in at least two types of impulsive behaviors or activities that may be damaging to oneself (such as reckless spending, unsafe sex, abusing substances, [binge eating](#))
5. Repeated threats of suicide, suicide attempts, or self-harm
6. Intense mood swings (like a period of depression, anxiety, and irritability that lasts for a few hours to a few days)
7. Continuously feeling empty
8. Difficulty keeping anger under control and sudden, inappropriate, and intense bouts of anger or aggression without cause
9. Feeling paranoid or disassociated from the world and oneself in stressful

situations

The variability of the DSM-5's criteria for BPD shows how this mental disorder can look very different from one individual to the next.

Experts have different opinions on whether there are distinct categories or “types” of BPD, and research reveals a similarly mixed bag.¹²

For example, Dr. Mondimore generally categorizes BPD cases by severity. “There’s a wide variation in severity such that some people with the problem are extremely impaired and can’t work, can’t maintain relationships, and sometimes can’t even support themselves,” Dr. Mondimore says. “And then there are other people who are functional and have less severe behavioral problems, so can be self-sufficient.”

What Is Paranoid Ideation?

Paranoid ideation is a symptom that can occur in borderline personality disorder, post-traumatic stress disorder (PTSD), and psychotic disorders such as schizophrenia. It involves transient, stress-related paranoia. Paranoia is characterized by the experience of feeling threatened, persecuted, or conspired against.¹ It can also loosely refer to beliefs of general suspicion regarding the motives or intentions of others.

If you have [borderline personality disorder \(BPD\)](#), it's likely that you have experienced transient paranoid ideation under stress.² It is one of the possible criteria for diagnosis, according to the current [Diagnostic and Statistical Manual of Mental Disorders \(DSM-5\)](#).

Symptoms

Symptoms of paranoid thinking can differ in terms of duration and severity. Some people may have very brief, mild paranoid ideations. For other people, these thoughts can be more severe and persistent. Some common symptoms include:

- Anxiety and stress
- Difficulty with relationships
- Distrust
- Feeling exploited
- Feeling isolated
- Feeling like a victim

- Feeling persecuted by others
- Interpreting body language, words, and glances as hostile
- Thinking that they are being watched or spied on

Paranoid ideation is not the same thing as delusional paranoia, which involves beliefs that are false and fixed.

For example, if you are experiencing delusional paranoia, you might have an ongoing belief that the government has bugged your house and car in order to keep tabs on you. Paranoid *ideation* may occur transiently. For example, you might see two people in the hallway talking and briefly believe they are talking about you.

Diagnosis

Paranoid ideation is one of the symptoms that might lead to the diagnosis of borderline personality disorder. In order to be diagnosed with borderline personality disorder, you must be evaluated by a qualified mental health professional.

While there is no definitive test for the disorder, these are some of the common signs and symptoms:³

- Anger issues, such as becoming extremely angry in inappropriate situations, exploding in rage, or being unable to control your anger, perhaps followed by feeling guilty or ashamed
- A perception of yourself that changes often and affects your thoughts, behaviors, opinions, relationships, and moods
- Extreme efforts to avoid real or perceived rejection or abandonment by others
- Feelings of disconnection with your body and/or your mind and paranoid thoughts that are brought on by stress
- Intense and unstable love-hate relationships with others
- Perpetual feelings of being bored and/or empty
- Risky, impulsive behavior, such as going on shopping sprees, using illicit drugs, or engaging in risky sex
- Suicidal behavior and/or behavior that's harmful to yourself
- Times of extreme emotion that last from a few hours to a few days and involve depression, anxiety, or irritability

If you are having suicidal thoughts, contact the [National Suicide Prevention Lifeline](#) at **1-800-273-8255** for support and assistance from a trained counselor. If you or a loved one are in immediate danger, call 911.

For more mental health resources, see our [National Helpline Database](#).

Causes

No one knows [what causes BPD](#) or the paranoid ideation that may accompany the condition. It's believed that environmental factors, genetics, and abnormalities of the brain may all be involved.

- **Changes in the brain:** There may be abnormalities in the brain that can lead to developing BPD. This is particularly true of areas of the brain that control emotions and judgment.
- **Childhood trauma:** Specifically, people with a history of child abuse, neglect, or other childhood trauma are more likely to have BPD.⁴
- **Family history:** If you have a parent or sibling with BPD, you are more likely to develop it yourself.
- **Interruptions in reasoning:** People are more likely to have paranoid thoughts when their ability to reason and interpret the world around them is compromised.⁵
- **Stress:** Stressful and traumatic events often play a role in triggering paranoid ideation.

Treatment

If you are having paranoid ideation due to BPD, it is important to find the appropriate treatment for your condition. Your treatment plan will likely involve a combination of medications and psychotherapy.²

- **Psychotherapy:** Common psychotherapies used to treat BPD are [dialectical behavioral therapy](#) (DBT), psychodynamic therapy, and [cognitive-behavioral therapy](#) (CBT).
- **Medication:** A combination of different medications may be used to help treat your symptoms as well. Typical medications include [antipsychotics](#), [antidepressants](#), and [mood stabilizers](#).

Coping

Paranoid ideation in BPD is usually triggered by stress. Finding ways to manage stress levels can be an effective way to cope with paranoid thoughts. Some strategies you might try include:

- **Deep breathing:** This is a common stress management technique that involves taking slow, deep breaths. It can aid in relaxation and ease feelings of anxiety. This technique is particularly useful because it can be used anywhere at any time.
- **Meditation:** This ancient technique has a wide range of health benefits, including easing stress and anxiety. There are a wide variety of meditative techniques, but mindfulness meditation, a strategy that involves focusing on the present, might be particularly helpful.
- **Progressive muscle relaxation (PMR):** This technique involves tightening and then relaxing various muscle groups throughout the body. After some practice, people can use PMR to quickly induce feelings of relaxation.
- **Regular exercise:** Physical exercise can be a great way to cope with feelings of stress. Research has also shown that exercise can help manage the body's reaction to anxiety.⁶
- **Visualization:** This technique involves using mental imagery to help induce a more relaxed state of mind. When you find yourself experiencing paranoid or panicked thoughts, for example, you might focus on an image that helps you feel calm and grounded.
- **Yoga:** This form of exercise can be a great way to combine physical activity with calming, meditative movements.

Additional Resources:

<https://damorementalhealth.com/does-my-loved-one-have-borderline-personality-disorder/>

<https://neuro.psychiatryonline.org/doi/full/10.1176/jnp.15.3.294>

Studies of the Associations Between Serotonergic Function and Aggression:

An early influential study²¹ found that “lifetime aggression” and suicide were associated with lower levels of CSF 5-HIAA in a group of 26 military men. Presence of personality disorder was an inclusion criterion for the study. The subjects had lifelong psychopathology and current difficulties adjusting to military life. “Lifetime aggression” was assessed through nine categories of behaviors, only two of which consisted of actual assaults or fighting. The authors state that the other seven categories were included because they are “related to history of aggressive behavior.” They included a history of temper tantrums, school discipline problems in childhood, antisocial behaviors, problems with getting along with supervisors, and military disciplinary problems in adulthood. The “aggression score” that was negatively related to serotonin levels included these problems. In another study by the same group,²² similar results were obtained. “Life history of aggression,” as defined above, was significantly associated with lower CSF 5-HIAA levels. The 12 subjects in this study were all diagnosed with borderline personality disorder and presented with lifelong problems in various areas of functioning. A history of suicide attempts was also associated with lower CSF 5-HIAA.

One study compared 16 murderers, 22 suicide attempters, and 39 controls.²⁴ The suicidal patients, but not the murderers, had significantly lower levels of CSF 5-HIAA than the controls. Though 15 of the 16 murderers were considered “impulsive,” they did not have lower serotonin levels. There was, however, a small subgroup of five subjects who differed from the other murderers in that they had killed their sexual partners. These subjects had very low levels of CSF 5-HIAA. The authors assume that these crimes were committed in a state of intense negative affect: jealousy, frustration, or fear. Thus, they conclude that serotonin dysfunction is associated with violence that occurs “in states of emotional turmoil.” This assumption, however, remains very questionable, given the very small sample size of the homicide group and the posthoc character of these analyses.

An important group of studies by Finnish and American collaborators emphasized that low serotonin function is associated specifically with “impulsive violence.” They defined the crime as impulsive when it was committed “without provocation or premeditation.” The first such study²⁵ investigated serotonergic function in 36 Finnish violent alcoholic offenders who had committed acts of “particular cruelty.” Low CSF 5-HIAA was found in the “impulsive” group (i.e., those who did not premeditate their criminal act), but not in the “nonimpulsive” group. The authors concluded that low CSF 5-HIAA concentration may be “indicative of impulsivity rather than violence.” Of the groups studied, impulsive violent offenders who had attempted suicide had the lowest 5-HIAA. In another such study,²⁶ impulsive and nonimpulsive violent alcoholic offenders and healthy volunteers were assessed. The impulsive offenders had lower CSF 5-HIAA than normal controls. The nonimpulsive violent patients had *higher* concentrations than both the impulsive offenders and the normal controls. No explanation was offered for the association between nonimpulsive violence and increased serotonergic function.

A number of problems exist with the definition of impulsivity as delineated in these studies. The violent crime was classified as impulsive when it occurred without provocation or premeditation, i.e., if the subject did not know the victim and investigators could not detect a rationale for the aggression during the psychiatric examination. The perpetrators themselves were then termed “impulsive” on the basis of the initial impulsive crime. A combination of two behaviors, such as externally directed violence and suicide, was interpreted as evidence of stronger impulsivity. Yet, violence occurring without identifiable provocation or premeditation may be indicative of greater psychopathology and not necessarily impulsivity. Differences other than impulsivity exist between the “impulsive” and “nonimpulsive” subjects that may account for the dissimilarity in the violent behavior. The “impulsive” subjects had more pervasive problems and were diagnosed with more severe disorders. In one of the above studies,²⁶ for example, all the impulsive subjects were diagnosed with either antisocial personality disorder or intermittent explosive disorder, and almost all (93%) received the additional diagnosis of borderline personality disorder. None of the nonimpulsive patients were diagnosed with antisocial personality disorder or intermittent explosive disorder, and only 27% were diagnosed with borderline personality disorder. Clearly, these subjects differ in areas other than the “impulsive” crime they committed; and that crime itself may be reinterpreted in the context of the overall pathology. One must also consider the impact of alcohol, as most subjects in these studies, impulsive, and nonimpulsive were alcoholics. Alcohol might have a different effect on the two groups.



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